

RESEARCH BRIEF

Topic: Investigating Medicaid Claim Denials

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Purpose: This guide explains Medicaid claim denial drivers, their impacts, and resolution workflows for onboarding staff and ensuring consistent billing operations.

Executive Summary

Timely Medicaid reimbursement relies on accurate documentation, compliance with payer requirements, and complete supporting records. When claims are denied, finding the root cause often involves gathering information from various systems and documents, reconciling discrepancies, and preparing evidence-based summaries for review by managed care organizations (MCOs).

Author's Role: As the Revenue Cycle Manager and Analyst, I regularly investigate Medicaid claim denials by analyzing payer responses, authorization records, client documentation, and billing history. This helps determine root causes, prepare supporting documents for claim reconsiderations, and recommend corrective actions.

Information Sources Analyzed

Source	Purpose
Medicaid Remittance Advice (EOB)	Identify specific denial and remark codes
MCO Portal	Validate authorizations and explanation of benefits
Client Records	Verify authorization details and service delivery dates
Case Manager Documentation	Confirm supporting documentation
Internal Billing Database	Identify historical claim patterns and recurring trends
Medicaid Eligibility Portals	Verify beneficiary coverage during specific dates of service
MCO Correspondence	Document official payer communications and formal resolutions

Research Objective

- Create a standardized investigation framework that enables staff to:
 - Consistently identify root causes of claim denials
 - Gather and validate supporting evidence from various systems
 - Prepare clear, evidence-based documentation for reconsiderations and appeals
 - Track and categorize recurring denial trends over time
 - Recommend operational process improvements to lower future denial rates
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Observed Denial Patterns

Analysis of recurring claim denials identified distinct patterns among the three Managed Care Organizations (MCOs) supported by the Revenue Cycle team. Denials were grouped according to their verified root cause rather than their initial surface-level denial code:

Category	Observed Frequency	Primary Cause	Responsibility
Prior Authorization	85%	Services rendered prior to authorization generation or unlinked 837P files	Shared (Provider / MCO)
Eligibility & Coverage	10%	Missed Medicaid renewals, or CHIP funding conflicts.	Client / Guardian
System Processing	3%	MCO-side processing glitches or clearinghouse submission errors.	Payer
Coding Errors	2%	Discrepancies in ICD-10 diagnosis or CPT procedure codes.	Provider

Operational Impact Breakdown:

- Prior Authorization (85%): Most denials resulted from timing lags between service delivery and MCO code generation. Additionally, recurring MCO-side authorization processing issues led to unacceptable denials, even when valid authorizations were on file.

- **Eligibility Issues (10%):** These were caused by coverage lapses when transitioning from private pay to Medicaid or by accepting out-of-county clients under CHIP (Children's Health Insurance Program) funding.
- **System Processing & Coding (5% combined):** Issues included system crashes during batch submissions and occasional manual entry errors with billing codes.

Research Process

Denial trends were identified through regular analysis of Medicaid remittance advice, MCO correspondence, authorization records, billing history, and internal client documents. Denials were categorized by root cause after verifying information across independent sources. Percentages reflect observed patterns over recurring billing cycles, not a formal statistical sample.

Recurring Authorization System Failures

Over multiple billing cycles, recurring authorization-related denials occurred even though valid authorizations were confirmed within the MCO's system. These denials differed from standard errors, affecting large batches of claims across multiple providers. This suggested a system-level glitch at the MCO rather than isolated billing errors.

- **Operational Impact:** During these incidents, 20% to 45% of submitted claims needed manual investigation, which delayed reimbursement.
 - **Resolution Workflow:** Claims were researched one by one, supporting documents were compiled, and structured findings were escalated to the MCO. Batch reconsideration requests were submitted and tracked until the MCO confirmed system corrections.
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Recommendations & Key Takeaways

Immediate Actions

- Cross-check provider authorizations before submitting claims.
- Log all reconsideration claim IDs in a central database for audit tracking.
- Verify receipt and storage of necessary client choice forms.

Long-Term Improvements

- Standardize authorization verification procedures across all MCO contracts.
- Track and log recurring denial patterns by specific payer.
- Develop internal checklists for documentation when onboarding staff.

Core Research Takeaway:

Effective denial resolution relies on validating information across various independent sources instead of just depending on surface-level denial codes. Root-cause analysis turns isolated billing errors into actionable insights that promote long-term process improvements.